

Optional Supplemental Care for Assisted Living Participants (OSCAP) Services Internal Policies and Procedures

The Optional Supplemental Care for Assisted Living Participants (OSCAP) service provides additional reimbursement to facilities that aid with personal care for Optional State Supplementation (OSS) residents who meet the medical criteria required for participation.

Confidentiality

Information about Medicaid applicants and recipients shall be safeguarded in accordance with the provisions of the Department of Health and Human Services regulations governing confidentiality and HIPAA (Regulations 126-170, et, seq., Refer to local CLTC office procedures). Confidential information includes, but is not limited to:

1. The name and address of the individual served, and the service received.
2. Medical data, including diagnosis and history of disease or disability.
3. Agency evaluation of information about a person.
4. The identity of persons or institutions that furnished health services to a person.
5. The social and economic conditions or circumstances of any person served.
6. Federal program information identified as confidential by the appropriate federal authority.

OSCAP Referral and Intake

Referral Sources for new OSCAP referrals

A referral to the Optional Supplemental Care for Assisted Living Participants (OSCAP) services may originate from sources such as:

- OSCAP enrolled CRCF (Community Residential Care Facility)

CRCFs with an OSCAP contract should refer all OSS residents to OSCAP. Due to the large volume of OSS residents in OSCAP contracted CRCFs, the SCDHHS regional nurse contacts the CRCF administrator prior to anticipated assessment completion to prompt the administrator of their future visit.

Referral Process

Referrals for the OSCAP service may be submitted electronically via the following link:
https://phoenix.scdhhs.gov/eref/electronic_referrals/new

Select reason for referral: OSCAP

To expedite the referral process, SCDHHS encourages electronic completion of referrals. By using electronic referral system, demographic information is captured earlier, which initiates the client assessment process.

Referrals may also be made at the county Medicaid eligibility office. Phone referrals will also be accepted utilizing the following number: Phone: 888-971-1637

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OSCAP Intake Criteria

The following criteria must be met for a participant to be considered appropriate for intake:

1. Must be an OSS participant
2. Residing in an OSCAP enrolled Community Residential Care Facility

If the OSCAP applicant is not verified by MMIS (pay category 85/86), the referral will be terminated. The CRCF can reapply once the resident is OSS approved.

When a completed referral is made to the OSCAP service, the case will be assigned to a SCDHHS Nurse Consultant for assessment completion.

Applicants who do not meet Intake Criteria

When a person does not meet OSCAP intake criteria, the application is closed and notated.

ASSESSMENT SCHEDULING PROCEDURES

1. Calls CRCF to schedule assessment visit.
2. Annotates the Phoenix narrative and notifies facility and OSS program manager, if an assessment visit is delayed or cancelled (by email)
3. Request a TAD for CRCF(s) 72 hours prior to scheduled visit. If the nurse consultant is planning to assess multiple residents at the CRCF
4. Request assessment assistance from other SCDHHS nurses when needed.

OSCAP POLICIES

Timeliness Standard for Completion of an Assessment

The Nurse Consultant must visit the resident and complete the assessment within ten (10) business days of the case assignment. A level of care is determined within five (5) business days after receiving all information needed. Any exception to these timeframes must be documented in the Phoenix narrative.

Assessment Completion

The assessment form must be completed accurately, obtaining all available information. Only pertinent information obtained and verified by the Nurse Consultant will be entered into the Phoenix System. Each time the assessment information is reevaluated the new information must

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be recorded in Phoenix. It is the Nurse Consultant's responsibility to ensure that the assessment is properly completed before a level of care decision is determined.

The assessment information may be obtained from a combination of the following sources: resident records and staff, family, and most importantly, the resident interview. Accurate observation can only be made when the Nurse Consultant sees and speaks to the resident. The Nurse Consultant must attempt to involve the resident as much as possible in the assessment process.

When a resident is physically or mentally unable to participate in an interview, a relative, guardian, responsible person or staff member can be interviewed for completion of the assessment. Narrate in Phoenix who the information was obtained from and include the title and/or relationship. The SCDHHS Nurse should try to complete the Mental Status Questionnaire (MSQ) initially to aide in the determination of the resident's mental awareness.

Medically Ineligible Residents

When an OSCAP applicant has been found to be medically ineligible, the case should be team staffed with another Nurse Consultant. The assessing Nurse Consultant should review the information that has been provided. All OSCAP assessments should have narratives including location, date, and staff on duty, and dependencies, medical conditions, impairments, and LOC.

Assessment Completion for the OSCAP Service

The assessment must be completed by a Nurse Consultant for a resident who applies for the OSCAP service. The Nurse Consultant will need:

- Copy of Medication Administration Report (MAR)/Physicians Orders
- Copy of Individual Care Plan (ICP) for the last six months
- Resident's chart
- Recent height, weight, and vital signs
- Interview with direct care staff
- Interview resident

When the resident has been determined to meet level of care, the service care plan is routed to the facility within five (5) business days of the assessment completion.

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Optional Supplemental Care for Assisted Living Participants Assessment Instructions:

OSCAP Level of Care

The Level of Care (LOC) determination is the process of identifying the extent of a person's cognitive impairments and functional dependencies. The OSCAP service utilizes the South Carolina Level of Care Criteria for Medicaid-Sponsored Long-Term Care as the foundation for determining level of care. The Criteria used primarily by CLTC have been adapted by OSCAP for determining medical necessity for OSCAP services.

Medical Necessity

Medical necessity determination for OSCAP services includes the following:

1. The person must have a cognitive impairment and one functional dependency, or
2. The person must have two functional dependencies.

Functional Dependency

A functional dependency is an inability to perform an activity of daily living (ADLs) independently, thereby requiring limited assistance from another person to perform the activity.

The seven functional areas of ADL are:

- Transferring
- Locomotion
- Bathing and personal grooming
- Dressing
- Eating
- Toileting and maintaining continence
- Incontinence Care

Cognitive Impairment

Persons are eligible for OSCAP services when they have at least one functional dependency and one or more of the following for cognitive impairment:

1. Supervision of moderate/severe memory loss, either long or short-term, manifested by disorientation, bewilderment, and forgetfulness, which requires significant intervention in overall care planning.
2. Supervision of moderately impaired cognitive skills manifested by decisions which may reasonably be expected to affect an individual's own safety.
3. Supervision of moderate problem behavior manifested by verbal abusiveness, physical abusiveness, or socially inappropriate/disruptive behavior, and frequent mood episodes.

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The Level of Care Determination Process

Once the Nurse Consultant determines that adequate information has been received to make a level of care decision, the assessment must be reviewed in Phoenix. A recommended level of care will be made by Phoenix. If the LOC determined by the Nurse Consultant is not what was recommended by Phoenix, the LOC must be team conferenced with another Nurse Consultant. Signatures of both nurses are required, and the conference annotated in the narrative by the assessing Nurse Consultant. If the team was unable to determine level of care, the team should confer with their supervisor. The supervisor will make the final LOC decision and act as the third signature.

The level of care and the justification for the level of care determination must be properly documented by the Nurse Consultant in the Phoenix. The Nurse Consultant should inform the referral source of the level of care using the OSCAP Status Forms. The Nurse Consultant emails the OSCAP Status Form to NEATO RSP group.

If the applicant is determined by the Nurse Consultant to meet intermediate LOC, and referral will be made in Phoenix for the Assisted Living Waiver service to CLTC by the Nurse Consultant.

Service Care Planning

The service plan is a document, developed in Phoenix, by the Nurse Consultant following the assessment of each OSS participant who meets the OSCAP or Intermediate Level of Care. It identifies the Activities of Daily Living, (ADL) dependencies for each participant.

The service plan is sent to the participating facility with the Service Authorization within five (5) business days of completing the assessment. The service plan will be mailed, emailed or faxed to the facility. A copy must be kept in the resident's file and available to any SCDHHS staff upon request.

The service plan will not be developed for those OSS participants determined to be medically ineligible, or for non-contracted facilities.

The service plan will be incorporated into the Individual Care Plan, (ICP), by the Facility Nurse, who will, also, create the Monthly Care Log for the Resident Aide who Documents direct care rendered.

- A service plan will be individualized for each OSCAP participant by the SCDHHS nurse.
- Service plans will be mailed, emailed, or faxed to the facility following the assessment and level of care determination.
- Service plan must be used as guidance to revise individual care plans and to create the Resident Monthly Task Log.
- A copy must be kept in the resident's file and available to any SCDHHS staff upon request

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Service Authorization/Termination

Authorization of OSCAP Services

Service authorization is the process of issuing a written document which enables the service provider's authorization for the participants into the OSCAP program. Implementation of the OSCAP service begins with the date on the signed authorization. The service plan date should be used for all authorization purposes.

OSCAP Services and Bed Holds

An interruption of OSCAP Services occurs for one of the following reasons:

Medical Bed Hold:

- The OSCAP resident enters the hospital or institution (rehabilitation Center or NH for a temporary stay of 30 days or less)

Non-Medical Bed Hold

- The OSCAP resident remains in the CRCF but does not receive the OSCAP services (resident visits family or refuses services)

The effective date of the interruption is the first date the service was not provided regardless of when the OSS/OSCAP program office is notified of the interruption. The interruption is documented on the monthly Census Log and the CRCF-01. The CRCF will not bill for OSCAP services on those days the resident is out of the facility or did not receive services. If services are not given/ received for more than 30 days, the case must be closed in Phoenix. Failure to inform the Nurse Consultant of interruption of services will be considered Medicaid fraud.

Transfer of an OSCAP Resident

OSCAP services will transfer with the resident to OSCAP enrolled providers. Providers are responsible for contacting the SCDHHS nurse to inform him or her of the transfer. If an OSCAP resident is transferred to a non-approved facility, the resident must be informed by staff of the discharging facility and allowed to choose another participating facility if desired. The authorization will be terminated, and the case closed in Phoenix, if the resident moves to a non-contracted CRCF.

Transfer of a Case to another Nurse Consultant

If a resident should transfer from one facility to another CRCF in a different region, the case will be transferred to the new area Nurse Consultant.

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****Participants may be reassigned to a Nurse Consultant for assistance in large facilities. The participants should be reassigned to the appropriate Nurse Consultant following the completion of assessment and level of care determination.**

Termination of OSCAP Services

Reasons for terminations of OSCAP residents:

- The resident no longer requires the service or if the resident becomes medically or financially ineligible to receive the service.
- If a facility has been terminated from the OSCAP Program for any reason, services must be terminated for each OSCAP resident.
- If a resident is transferred from one CRCF to another facility. A new referral should be made by the admitting facility if resident is transferred to a participating CRCF. If the admitting facility is not a participating OSCAP CRCF OSCAP services will be terminated.
- If a resident leaves the CRCF for more than 30 days such as returning home or admission to a nursing home the participant will no longer be eligible for OSCAP services and will be terminated.
- Death
- If a resident exceeds SCDHEC guidelines for determining appropriate CRCF placement and no effort is made to seek alternative placement by the CRCF.
- If a resident is receiving Hospice services

Authorization of OSCAP Services

- Fill in the appropriate information on the Authorization form including social security number and Medicaid number.
- Mails/Faxes a copy of the service authorization and service plan to the CRCF and email a copy of the authorization to the Program Assistant at Neatorsp@scdhhs.gov
- Places a copy of these documents in the resident's file

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Termination of Services Procedures

If the nurse consultant is informed of the termination the following steps should occur:

- An OSCAP status form should be sent to the facility.
- Fax/mail/email service termination to CRCF and program assistant, if applicable.
- If the participant is found to be ineligible for the program but still resides in the facility with the OSS rate the participant is reassigned to the program supervisor. The participant status in Phoenix should be changed to “medically ineligible.”
- OSS only participants in facilities not participating in OSCAP will no status changes and no notifications sent.
- Narrate in Phoenix the reason for termination of services

CRCF that Closes or no Longer OSCAP Program Participant

- Termination forms will be sent to the involved facility with copies to the OSS/OSCAP program coordinator and the resident/responsible party.
- Narrate in Phoenix
- For each resident indicate the reason for termination

Note: This procedure will also apply when a facility’s license is suspended or revoked and/or when the OSS enrollment or IPC contract is terminated.

Phoenix Monitoring

Nurse consultants must monitor their caseload using their Phoenix dashboard monthly to ensure the OSCAP participants are pay category 85 or 86, and financially eligible. If the participant, no longer meets the qualification the information should be narrated in Phoenix and the OSCAP case closed.

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Timeliness Chart

- Assessment within ten (10) business days of the case assignment.
- A level of care is determined within five (5) business days after receiving all information needed.
- The service plan is sent to the participating facility with the Service Authorization within five (5) business days of completing the assessment.
- Re-evaluate OSCAP participants every 18 months or soon if level of care changes.